

BEAR VALLEY COMMUNITY HEALTHCA 41870 GARSTIN DR

BIG BEAR LAKE CA 92315

EMERGENCY ROOM • OUTPATIENT RECORD

SUB TYPE	SERVICE	EXPECT DATE	CITIZENSHIP										
ER		6/30/24											
PATIENT NUMBER	TYPE	PATIENT NAME	AGE	BIRTHDATE	SEX	M/S	DATE OF SERVICE	TIME	CLERK INIT.				
10279723	3	BEGLE PETER	87	03/19/1937	M	SW	6/30/24	17:36	JSS				
ADDRESS - LINE 1		ADDRESS - LINE 2		CITY		STATE	ZIP CODE	TELEPHONE					
PO BOX 554				FAWNSKIN		CA	92333	909-800-5237					
PATIENT SSAN	NOTIFY IN CASE OF EMERGENCY - NAME		RELATIONSHIP		ADDRESS		TELEPHONE						
	BEGLE PETER E		SON				619-253-5324						
INSURANCE COMPANY			CONTRACT OR GROUP NUMBER		DATE		PLACE		ACCIDENT				
SR HERITAGE/VICTOR VALLEY IPA			H73658492		6/30/24		Other Accident						
					TIME		EVENT						
					17:00								
GUARANTOR NAME		GUARANTOR ADDRESS		CITY		ZIP CODE		GUAR. TELEPHONE					
BEGLE PETER		PO BOX 554		FAWNSKIN		CA 92333		909-800-5237					
GUARANTOR EMPLOYER		GUARANTOR OCCUPATION		GUAR. EMPLOYER ADDRESS						GUAR. EMP. TELEPHONE			
PREV. SERVICE	PREV. SERV. DATE		IF MINOR - PARENT NAME		MED. REC. #		ATTENDING/2ND PHYSICIAN						
10273680	3/29/24				123464		RILEY HOYE/						
CHARGES		X-RAY	LAB	RESP. TH.	PHY. TH.	EKG	I.V.	DRUGS	SUPPLIES	OTHER	M.D.	E.R. RM	TOTAL DUE

AUTHORIZATION FOR TREATMENT, GUARANTEE OF PAYMENT, ASSIGNMENT OF INSURANCE BENEFITS

- The undersigned has been informed of the emergency treatment considered necessary for the above named patient, and that treatment and procedures will be performed by physicians, members of house staff and employees of the hospital. Authorization is hereby granted for such treatment and procedures. The undersigned has read the above authorization and understands the same and certifies that no guarantee or assurance has been made as to the results that may be obtained.
- The undersigned agrees to pay for services rendered by Hospital upon release of patient.
- I/we hereby assign any hospital benefits, sick benefits, injury benefits due to a liability of a Third party, payable by any party, for the above patient, to Hospital unless I pay the account in full upon release of patient.
- I/we hereby authorize the "Administrator of Hospital" to furnish from its records any information requested by the before mentioned insurance companies in connection with the above assignment. I do hereby appoint the "Controller" of Hospital as my lawful attorney to endorse for me any checks made payable to me for benefits or claims collected under the above assignment and to apply any credit balance to any other account I may owe said hospital.

DATE	TIME	SIGNED PATIENT	SIGNED GUARANTOR
CHIEF COMPLAINT (If Accident State How, When, and Where)			ADVANCED DIRECTIVE
PELVIC PAIN POST TC			Y

TEMP.	PULSE	RESP.	B/P	ALLERGIES	MEDICATIONS - HOME	E.R. PHYSICIAN	TET. TOX.
				Codei			

NURSES NOTES:

LAB DATA (Including X-Rays, EKGs, etc.)	NURSE'S SIGNATURE (RN OR LPN)

PHYSICIAN'S REPORT

DIAGNOSIS:

TREATMENT:

INSTRUCTIONS TO PATIENT:

CONDITION ON DISC		
IMP	STABLE	EXPIRED

FOLLOW-UP WITH

M.D.

BY SIGNING HERE I CERTIFY THAT I UNDERSTAND THE FOLLOW-UP INSTRUCTIONS RECEIVED BY ME IN WRITING, WHICH WERE EXPLAINED TO ME.

DATE - TIME OF DISC.

PHYSICIAN'S SIGNATURE

M.D.



EMERGENCY DEPARTMENT VISIT OVERVIEW

Bear Valley Community Health District, Big Bear Lake CA

6/30/2024 9:52 PM

This is a preliminary document and is subject to change.

Patient: BEGLE, PETER	Med Rec #: 123464	Isolation Status:
Age / DOB / Sex: 87 yr, 03/19/1937, M	HIS Visit ID: 10279723	Adv Directive: No
ED Arrival: 17:36 06/30/2024	Ethnicity:	Language:
Diagnosis:		

CHIEF COMPLAINT

- MOTOR VEHICLE COLLISION.

ALLERGIES

- NKDA

MEASUREMENTS

Height	5 ft 4 in	162.5 cm
Weight	143.3 lb	65.0 kg
BMI	24.6 kg/m2	

PAST MEDICAL HISTORY / PROBLEMS

- Contusion
- Dizziness
- Fall
- Glaucoma
- Hypercholesterolemia
- Muscle Strain, Lower Extremity
- Neuropathy
- Prostate Cancer
- Weakness

PAST SURGICAL HISTORY

- Hip Prosthesis
- Hip Surgery

SOCIAL HISTORY

- Smoking status: no
- Alcohol use: no
- Drug use: no
- infectious dz exposure: no
- Reports abuse: no
- Self-harm assessment: no risk
- Fall assessment: no
- Nutritional assessment: no deficits
- Functional assessment: no impairment identified

SOCIAL HISTORY

- Learning needs: no barriers
- Recent travel: no

HOME MEDICATIONS

- None

SOURCE FOR HOME MEDICATIONS

ED COURSE

MEDICATIONS GIVEN IN EMERGENCY DEPARTMENT

18:31 06/30/24	ANCEF [IVPB] (CEFAZOLIN SODIUM) 2 gm 50 mL/hr IVPB in site #1 left forearm
18:37 06/30/24	TDAP [IM] 0.5 mL IM
19:50 06/30/24	Xylocaine Viscous * 10 * Topical
19:51 06/30/24	LIDOCAINE 1% WITH EPINEPHRINE [INJECTION] 20 mL Injection
21:21 06/30/24	GABAPENTIN [PO] 300 mg PO

IV SITE INFORMATION

17:46 06/30/24 Site # 1 left forearm, 18g.

INTAKE

OUTPUT

REASSESSMENT (most recent)

- 17:49 06/30/2024
RESPIRATORY: Respirations not labored.
SKIN: Skin is warm and dry.

VITAL SIGNS

Vital	Time	FIRST	Time	LAST
Temp	17:46 06/30/24	98	17:46 06/30/24	98
BP	17:46 06/30/24	160/80	17:46 06/30/24	160/80
HR	17:46 06/30/24	64	17:46 06/30/24	64

[] Indicates typed, keypad and/or cut & pasted entry



EMERGENCY DEPARTMENT VISIT OVERVIEW

Bear Valley Community Health District, Big Bear Lake CA

6/30/2024 9:52 PM

This is a preliminary document and is subject to change.

Patient: BEGLE, PETER	Med Rec #: 123464	Isolation Status:
Age / DOB / Sex: 87 yr, 03/19/1937, M	HIS Visit ID: 10279723	Adv Directive: No
ED Arrival: 17:36 06/30/2024	Ethnicity:	Language:
Diagnosis:		

VITAL SIGNS

Vital	Time	FIRST	Time	LAST
RR	17:46 06/30/24	19	17:46 06/30/24	19
O2 Sat	17:46 06/30/24	94	17:46 06/30/24	94
Pain	17:46 06/30/24	1	17:46 06/30/24	1

PROCEDURES

- Wound Repair

NURSING INTERVENTIONS

LABS / STUDIES

LABS / STUDIES ORDERED

- APTT - Done
- BB Type & Screen -
- CBC w Diff - Done
- Chest 1V Port - Done
- CMP - Done
- CT ABD/PEL w IV Cont - Done
- CT C-Spine wo IV Cont - Done
- CT Head wo IV Cont - Done
- CT L-Spine wo IV Cont - Done
- Femur R 2V - Done
- Hand R 3V - Done
- Hip Limited 1-2V Right - Done
- Lactic Acid (Lab) - Done
- Lipase - Done
- Pelvis AP - Done
- Prothrombin Time (PT) - Done
- Urinalysis - Ack
- Wrist 3+V Right - Done

ABNORMAL LAB RESULTS

- APTT
PTT: 24.00 (L)

ABNORMAL LAB RESULTS

- CBC w Diff
RBC: 4.43K/uL (L)
MCH: 32.1pg (H)
%NEUT: 77% (H)
%LYMPH: 16% (L)
- CMP
CHLORIDE: 108mEq/L (H)
ANION GAP: 10mEq/L (L)
BUN: 27mg/dL (H)
- Lipase
LIPASE: 386U/L (H)

LABS PENDING IMPORT

- APTT
- BB Type & Screen
- CBC w Diff
- Chest 1V Port
- CMP
- CT ABD/PEL w IV Cont
- CT C-Spine wo IV Cont
- CT Head wo IV Cont
- CT L-Spine wo IV Cont
- Femur R 2V
- Hand R 3V
- Hip Limited 1-2V Right
- Lactic Acid
- Lipase
- Pelvis AP
- Prothrombin Time (PT)
- Urinalysis
- Wrist 3+V Right

[] Indicates typed, keypad and/or cut & pasted entry



EMERGENCY DEPARTMENT VISIT OVERVIEW

Bear Valley Community Health District, Big Bear Lake CA

6/30/2024 9:52 PM

This is a preliminary document and is subject to change.

Patient: BEGLE, PETER	Med Rec #: 123464	Isolation Status:
Age / DOB / Sex: 87 yr, 03/19/1937, M	HIS Visit ID: 10279723	Adv Directive: No
ED Arrival: 17:36 06/30/2024	Ethnicity:	Language:
Diagnosis:		

CLINICAL IMPRESSION

- [1. MOTOR VEHICLE ACCIDENT, ACUTE
2. RIGHT COMMUNUTED ILIAC WING FRACTURE
3. L5 COMPRESSION, ACUTE
4. RIGHT ILIACUS HEMATOMA, ACUTE
5. RIGHT GLUTEAL STRAIN HEMATOMA]

DIAGNOSES PRESENT ON ADMISSION

[] Indicates typed, keypad and/or cut & pasted entry

Physician Clinical Report
Bear Valley Community Health District
Emergency Department
41870 Garstin Drive, Big Bear Lake, CA 92315 909-866-6501

Patient: BEGLE, PETER
MRN: 123464 Acct#: 10279723
Sex: M DOB: 03/19/1937 Age: 87y
Arrival: 06/30/2024 17:36 Departure: Disposition: Transfer

Weight: 65 kg. Height/Length: 64 inches. BMI: 24.6

***This is a preliminary document and is subject to change**

Time Seen: 17:52 06/30/2024.

Arrived- By ambulance. Historian- patient. Disposition decision: 20:58 06/30/2024.

HISTORY OF PRESENT ILLNESS

Chief Complaint: MOTOR VEHICLE COLLISION. The injury occurred just prior to arrival. Occurred on a street.

The patient complains of mild pain. No blow to the head, neck pain or loss of consciousness.

Mechanism details: Patient was driving the vehicle and was wearing a lap belt. Impact was on the front of the vehicle. The air bag deployed. The crash involved two vehicles and resulted in moderate damage to the patient's vehicle and accident involved a moderate impact velocity and estimated speed of the collision: 35 mph.

Additional history - (Peter is a 87 yo male who presents to the ED via EMS after being a restrained driver during a MVC. Patient was travelling approximately 35 mph when he struck another car head on. Airbags deployed and he was extricated from the car by EMS due to R hip pain. Patient denies LOC or hitting his head and can recall the event. Patient states the skin on his right hand was torn from the steering wheel. Patient denies back pain, headache, SOB, numbness/tingling of extremities or other joint pains.).

REVIEW OF SYSTEMS

All pertinent positives and negatives in HPI. All other systems reviewed and are negative.

PAST HISTORY

Peripheral Neuropathy
Glaucoma.

Medications:

None.

Allergies:

Patient: BEGLE, PETER

MRN: 123464

VisitID: 10279723

909-866-6501

87y, M

Physician Clinical Report

Bear Valley Community Health District

41870 Garstin Drive, Big Bear Lake, CA 92315

Registration Date/Time: 06/30/2024 17:36

NKDA.

SOCIAL HISTORY

Never smoker. No alcohol use or drug use. No recent travel.

ADDITIONAL NOTES

The nursing notes have been reviewed.

PHYSICAL EXAM

Vital Signs: 06/30/2024 17:46 BP: 160/80. MAP: 106. HR: 64. RR: 19. O2 saturation: 94%. Temp: 98.0 F. Have been reviewed. Hypertensive.

Appearance: Alert. Oriented X3. No acute distress.

Head: Head non-tender. No swelling of head.

Eyes: Pupils equal, round and reactive to light. EOM intact.

ENT: No dental injury. Pharynx normal.

Neck: Painless ROM. Non-tender.

CVS: Heart sounds normal. Pulses normal.

Respiratory: Painless inspiration. Breath sounds normal. Chest nontender.

Abdomen: No visible injury. Soft and nontender. No mass.

Back: No tenderness. ROM normal. No vertebral point tenderness.

Skin: Skin warm and dry. (Superficial abrasion on L elbow and L knee. Deep laceration with exposed tendons on the anterior R hand.).

Extremities: Mild bony tenderness present in the right hip. Left elbow: small abrasion with controlled bleeding. Right hand: deep 4.0 cm laceration with controlled bleeding. Pelvis stable. Right hip. Limited ROM secondary to pain. Left knee: small abrasion with controlled bleeding. No lower extremity edema. (Full passive ROM of R hip. 4/5 strength in R hip. Sensation intact in extremities.).

Gait: Gait not tested due to pain.

Neuro: Oriented X 3. No motor deficit. No sensory deficit.

PROGRESS AND PROCEDURES

Laceration Repair: Time: 20:21 06/30/2024. Location: (right hand). Time-out completed immediately before the procedure. Length: 3 cm. Complexity: complex (requiring extensive undermining) and simple (local anesthesia used and sutured).

Wound depth/shape- subcutaneous, irregular and with multiple flaps, with avulsion and involving fascia and muscle. Wound is clean. Distal neuro/vascular/tendon status normal. Sensory, motor and vascular deficit present distally. Tendon examined. No ligament or tendon injury or tendon deficit or laceration. Local and topical anesthesia provided using 2% lidocaine. Prepped with Betadine.

Wound explored and irrigated extensively with normal saline. Closure of superficial layer: interrupted 4-0 nylon (11 sutures). Steri-strips used. Post-procedure: he is stable and there are no complications. Bleeding is controlled and neuro-vascular status is intact distal to the wound. Dressing applied. Tetanus immunization given. Estimated blood loss: 3 mL. (Wound Repaired by NP Kelly Gora).

Course of Care: 87-year-old male with history of glaucoma, hyper cholesterolemia and peripheral neuropathy currently on gabapentin presents emergency department with right-sided hip pain and lower back pain after motor vehicle accident. This patient was in multi-vehicle motor vehicle accident. Patient was restrained driver airbags deployed. Traveling approximately 30 miles an hour. Front impact. Full recall of event. Patient was assisted at the scene by 1st responders out of his vehicle. Patient did ambulate briefly at the scene. EMS provided transport. Upon arrival patient's vital signs were reviewed his vital signs are interpreted as within normal limits for age. His primary survey was

unremarkable. Secondary survey was remarkable for right-sided hip pain limited right hip and movement and range of motion due to pain. And lumbar spine tenderness to palpation in the midline. He was also found to have a right dorsal hand laceration and skin tear on secondary survey. This patient had no evidence of head or neck trauma. He denied any neck pain. No other extremity injuries. No reported chest pain or shortness of breath to suggest acute chest injury. The patient's portable chest x-ray was without acute pathology. Portable chest x-ray without acute pathology including rib fracture or pneumothorax. AP pelvis without acute pathology, right total hip arthroplasty without acute abnormality or dislocation. Patient had imaging to include noncontrast head CT and CT C-spine that were without acute pathology. CT lumbar spine remarkable for L5 compression fracture with a minimal retropulsion. No neurologic deficits in the lower extremities. Patient's CT abdomen and pelvis was remarkable for right iliac wing fracture comminuted, nondisplaced. Right iliacus and right gluteal muscle hematoma and sacral ala fracture. Sheet placed around pelvis for fracture stabilization. Considering the patient's multiple injuries from today's motor vehicle accident plan for transfer of care to higher level of care with Trauma Services, inpatient medicine, physical therapy occupational therapy and Orthopedics.

Voice dictation software used for documentation. May contain grammar, spelling, syntax errors. Social Determinants of Care: Rural Environment, Critical Access hospital, Limited Access to Care.

case discussed with low malignant a medical Center Dr. Allison. He accepted transfer of care. Considering pelvic fracture with hematoma will prioritize transport and expedite with air transport, Mercy air.

21:02. discussed CT imaging finding with patient and the patient's daughter. Patient will require transfer of care to higher level of care with Trauma Services. patient is voluntary for transport.

Critical care performed (60 minutes). Time is exclusive of separately billable procedures. Time includes: direct patient care, patient reassessment, coordination of patient care, interpretation of data (laboratory data, pulse oximetry and chest xrays), review of patient's medical records, medical consultation, family consultation regarding treatment decisions and documentation of patient care-see progress notes. Procedures included in critical care time: peripheral IV placement and phlebotomy.

Disposition: Benefits, risks and alternatives to transfer explained to patient and daughter. Transferred to Loma Linda University Medical Center. Summary of care provided to transport team, patient, family and transfer facility via paper.

CLINICAL IMPRESSION

1. Motor Vehicle Accident, Acute
2. Right Comminuted Iliac Wing Fracture
3. L5 Compression, Acute
4. Right Iliacus Hematoma, Acute
5. Right Gluteal Strain Hematoma.

Hoyer, Riley, MD

Patient: BEGLE, PETER
MRN: 123464
VisitID: 10279723
909-866-6501
03/19/1937, 87y, M

OrderSheet
Bear Valley Community Health District
41870 Garstin Drive, Big Bear Lake, CA 92315

Registration Date/Time: 06/30/2024 17:36

WEIGHT:65 kg

ALLERGIES: NKDA

CHIEF COMPLAINT: MVC

LAB ORDERS

Order Description	Priority	Entered	Acknowledged	Initialed
CBC w Diff	STAT	17:50 06/30/2024 Hoyer, Riley MD;		18:10 Zapotosk Michael Zapotosky R.N.
CMP	STAT	17:50 06/30/2024 Hoyer, Riley MD;		18:10 Zapotosk Michael Zapotosky R.N.
Lipase	STAT	17:50 06/30/2024 Hoyer, Riley MD;		18:10 Zapotosk Michael Zapotosky R.N.
Prothrombin Time (PT) (No)	STAT	17:50 06/30/2024 Hoyer, Riley MD;		18:10 Zapotosk Michael Zapotosky R.N.
APTT (No)	STAT	17:50 06/30/2024 Hoyer, Riley MD;		18:10 Zapotosk Michael Zapotosky R.N.
Lactic Acid (Lab)	STAT	17:50 06/30/2024 Hoyer, Riley MD;		18:10 Zapotosk Michael Zapotosky R.N.
Urinalysis	STAT	17:50 06/30/2024 Hoyer, Riley MD;	Ack'd: 18:10 Zapotosky, Michael Zapotosky R.N.	
BB Type & Screen	STAT	21:33 06/30/2024 Hoyer, Riley MD;		

DIAGNOSTIC STUDY ORDERS

Order Description	Priority	Entered	Acknowledged	Initialed
-------------------	----------	---------	--------------	-----------

Patient: BEGLE, PETER
MRN: 123464
VisitID: 10279723
909-866-6501
87y, M

OrderSheet
Bear Valley Community Health District
41870 Garstin Drive, Big Bear Lake, CA 92315

Registration Date/Time: 06/30/2024 17:36

CT Head wo IV Cont	STAT	17:50 06/30/2024 Hoyer, Riley MD;	Ack'd: 18:10 Zapotosky, Michael Zapotosky R.N.	19:07 Hoogendyk, Kel
Reason for Study: Headache				
CT C-Spine wo IV Cont	STAT	17:50 06/30/2024 Hoyer, Riley MD;	Ack'd: 18:10 Zapotosky, Michael Zapotosky R.N.	19:07 Hoogendyk, Kel
Reason for Study: Pain				
CT ABD/PEL w IV Cont	STAT	17:50 06/30/2024 Hoyer, Riley MD;	Ack'd: 18:10 Zapotosky, Michael Zapotosky R.N.	19:07 Hoogendyk, Kel
Reason for Study: Abdominal Pain				
Chest 1V Port	STAT	17:50 06/30/2024 Hoyer, Riley MD;		18:10 Zapotosk Michael Zapotosky R.N.
Reason for Study: Shortness of Breath				
Pelvis AP	STAT	17:50 06/30/2024 Hoyer, Riley MD;	Ack'd: 18:10 Zapotosky, Michael Zapotosky R.N.	19:07 Hoogendyk, Kel
Reason for Study: Pain				
CT L-Spine wo IV Cont	STAT	18:17 06/30/2024 Hoyer, Riley MD;	Ack'd: 18:17 Zapotosky, Michael Zapotosky R.N.	19:07 Hoogendyk, Kel
Reason for Study: Lower Back Pain				
Hand R 3V	STAT	18:17 06/30/2024 Hoyer, Riley MD;	Ack'd: 18:18 Zapotosky, Michael Zapotosky R.N.	18:46 Zapotosk Michael Zapotosky R.N.
Reason for Study: Hands Pain				
Wrist 3+V Right	STAT	18:17 06/30/2024 Hoyer, Riley MD;	Ack'd: 18:18 Zapotosky, Michael Zapotosky R.N.	18:46 Zapotosk Michael Zapotosky R.N.

Reason for Study: Pain				
Femur R 2V	STAT	18:21 06/30/2024 Hoyer, Riley MD;	Ack'd: 18:35 Zapotosky, Michael Zapotosky R.N.	19:07 Hoogendyk, Kel
Reason for Study: Pain				
Hip Limited 1-2V Right	STAT	18:21 06/30/2024 Hoyer, Riley MD;	Ack'd: 18:35 Zapotosky, Michael Zapotosky R.N.	19:07 Hoogendyk, Kel
Reason for Study: Hip Pain				

MEDICATION/IV/DRIP/FLUID ORDERS

Order Description	Priority	Entered	Acknowledged	Initialed
IV Saline Lock		17:50 06/30/2024 Hoyer, Riley MD;		18:11 Zapotosk Michael Zapotosky R.N.
Ancef IVPB 2 gm (NOW)		18:19 06/30/2024 Hoyer, Riley MD;	Ack'd: 18:20 Zapotosky, Michael Zapotosky R.N.	18:36 Zapotosk Michael Zapotosky R.N.
Tdap IM 0.5 mL (NOW)		18:19 06/30/2024 Hoyer, Riley MD;	Ack'd: 18:20 Zapotosky, Michael Zapotosky R.N.	18:37 Zapotosk Michael Zapotosky R.N.
Xylocaine Viscous PO 10 mL (NOW)		19:27 06/30/2024 Hoyer, Riley MD;		19:51 Fogle, Joshua R.N.
Lidocaine 1% with Epinephrine Injection 10 mL (place at bedside)		19:27 06/30/2024 Hoyer, Riley MD;		19:51 Fogle, Joshua R.N.
Gabapentin PO 300 mg (NOW)		21:14 06/30/2024 Hoyer, Riley MD;		21:21 Fogle, Joshua R.N.

GENERAL ORDERS

Order Description	Priority	Entered	Acknowledged	Initialed
Cardiac Monitor (continuous)		17:50 06/30/2024 Hoyer, Riley MD;		18:10 Zapotosk Michael Zapotosky R.N.

Irrigate Wounds		19:27 06/30/2024 Hoyer, Riley MD;		19:51 Fogle, Joshua R.N.
Suture Set-up: (Nylon 4-0) (Nylon 5-0) (Irrigation Solution, Betadyne) (suture tray at bedside)		19:27 06/30/2024 Hoyer, Riley MD;		19:51 Fogle, Joshua R.N.

This document has not been locked and should not be saved in the medical record

Patient: BEGLE, PETER
MRN: 123464
VisitID: 10279723
909-866-6501
87y, M

Labs
Bear Valley Community Health District
41870 Garstin Drive, Big Bear Lake, CA 92315
Registration Date/Time: 06/30/2024 17:36

Femur R 2V: (COLL: 06/30/2024 18:21)
results

(MsgRcvd 06/30/2024 19:23) Final

Test
FEMUR RT

Result

Flag

(Reference)

Bear Valley Community Hospital
Radiology Department
Box 1649 - 41870 Garstin Drive
Big Bear Lake, CA 92315
909-878-8204 ? Fax: 909-878-8280

NAME: BEGLE, PETER
DOB: 3/19/37

DATE OF EXAM: 6/30/24
Patient No: 123464
Extern Acct: E10279723
Accsn: E500261820240630

Physician: HOYER, RILEY

EXAM: FEMUR RT

CLINICAL INDICATION: PAIN

TECHNIQUE: 4 radiographic views of the right femur were obtained.

Comparison: None

FINDINGS/IMPRESSION:

There is no evidence of acute fracture or dislocation. Status post total right hip arthroplasty with intact appearing orthopedic hardware. Vascular calcifications.

The visualized joint space is well maintained.

Mild degenerative changes of the knee joint.

There is no radiopaque foreign body.

Electronically signed by Sadat, Sayed at 06/30/2024 07:20 PM PST

Hip Limited 1-2V Right: (COLL: 06/30/2024 18:21)
results

(MsgRcvd 06/30/2024 19:23) Final

Test
HIP UNILATERAL LIMITED RT

Result

Flag

(Reference)

Patient: BEGLE, PETER
MRN: 123464
VisitID: 10279723
909-866-6501
87y, M

Labs
Bear Valley Community Health District
41870 Garstin Drive, Big Bear Lake, CA 92315
Registration Date/Time: 06/30/2024 17:36

Bear Valley Community Hospital
Radiology Department
Box 1649 - 41870 Garstin Drive
Big Bear Lake, CA 92315
909-878-8204 ? Fax: 909-878-8280

NAME: BEGLE, PETER
DOB: 3/19/37

DATE OF EXAM: 6/30/24
Patient No: 123464
Extern Acct: E10279723
Accsn: E500261920240630

Physician: HOYER, RILEY

EXAM: HIP UNILATERAL LIMITED RT

CLINICAL INDICATION: HIP PAIN

TECHNIQUE: 2 radiographic views of the right hip were obtained.

Comparison: HIP COMPLETE 2V RT on DOS: 9/14/23, HIP COMPLETE 2V RT on DOS: 2/28/24

FINDINGS/IMPRESSION:

There is no evidence of acute fracture or dislocation. Status post total right hip arthroplasty.

Intact orthopedic hardware. Vascular calcifications.

The visualized joint space is well maintained.

The alignment is anatomical.

There is no radiopaque foreign body.

Electronically signed by Sadat, Sayed at 06/30/2024 07:21 PM PST

CT L-Spine wo IV Cont: (COLL: 06/30/2024 18:17) (MsgRcvd 06/30/2024 20:20)
Correction to results

Exam
CT L-SPINE W/O CONTRAST

Bear Valley Community Hospital
Radiology Department
Box 1649 - 41870 Garstin Drive
Big Bear Lake, CA 92315
909-878-8204 ? Fax: 909-878-8280

NAME: BEGLE, PETER
DOB: 3/19/37

DATE OF EXAM: 6/30/24
Patient No: 123464
Extern Acct: E10279723
Accsn: E500261520240630

Physician: HOYER, RILEY

EXAM: CT L-SPINE W/O CONTRAST

INDICATION: LOWER BACK PAIN- S/P TC

COMPARISON: None

TECHNIQUE: Multiple axial CT images of the lumbar spine were obtained using bone algorithm. Axial and coronal reformatting was done. Bone and soft tissue windows were

reviewed.

Radiation Dose Information:

CT Dose: CTDI volume is 12.3 mGy. Dose-length product is 378.5 mGy*cm

FINDINGS:

Fracture of the superior endplate of L5 with minimal approximately 2 mm retropulsion. There is approximately 25% height loss . Partially visualized bilateral renal cysts. Grade 1 retrolisthesis of L5 in relation L4.

The disc spaces are relatively preserved. There is multilevel degenerative change of the spine, with disc space narrowing, subchondral sclerosis, and marginal osteophyte formation. Degenerative changes of the bilateral SI joints which include osteophytosis and osseous fusion.

IMPRESSION:

1. Acute wedge-shaped compression deformity involving the anterior vertebral body at L5 with approximately 25% height loss and minimal retropulsion.

2. Radiation optimization: All CT scans at this facility use at least one of these dose optimization techniques: automated exposure control mA and/or kV adjustment per patient size (includes targeted exams where dose is matched to clinical indication) or iterative reconstruction.

Electronically signed by Sadat, Sayed at 06/30/2024 08:18 PM PST

Hand R 3V: (COLL: 06/30/2024 18:17)
results

(MsgRcvd 06/30/2024 19:20) Final

Test
HAND 3+VWS RT

Result

Flag

****(Reference)****

Bear Valley Community Hospital
Radiology Department
Box 1649 - 41870 Garstin Drive
Big Bear Lake, CA 92315
909-878-8204 ? Fax: 909-878-8280

NAME: BEGLE, PETER
DOB: 3/19/37

Physician: HOYER, RILEY

EXAM: HAND 3+VWS RT

DATE OF EXAM: 6/30/24
Patient No: 123464
Extern Acct: E10279723
Accsn: E500261620240630

CLINICAL INDICATION: HANDS PAIN

TECHNIQUE: 3 radiographic views of the right hand were obtained.

Comparison: None

FINDINGS/IMPRESSION:

There is no evidence of acute fracture or dislocation.

Mild-to-moderate degenerative changes.

The alignment is anatomical.

There is no radiopaque foreign body.

Electronically signed by Sadat, Sayed at 06/30/2024 07:18 PM PST

Wrist 3+V Right: (COLL: 06/30/2024 18:17)
results

(MsgRcvd 06/30/2024 19:21) Final

Test
WRIST 3+VWS RT

Result

Flag

****(Reference)****

Bear Valley Community Hospital
Radiology Department
Box 1649 - 41870 Garstin Drive
Big Bear Lake, CA 92315
909-878-8204 ? Fax: 909-878-8280

NAME: BEGLE, PETER
DOB: 3/19/37

DATE OF EXAM: 6/30/24
Patient No: 123464
Extern Acct: E10279723
Accsn: E500261720240630

Physician: HOYER, RILEY

EXAM: WRIST 3+VWS RT

CLINICAL INDICATION: PAIN

TECHNIQUE: 3 radiographic views of the right wrist were obtained.

Comparison: None

FINDINGS/IMPRESSION:

There is no evidence of acute fracture or dislocation.

The visualized joint space is well maintained.

The alignment is anatomical.

There is no radiopaque foreign body.

Electronically signed by Sadat, Sayed at 06/30/2024 07:19 PM PST

CBC w Diff: (COLL: 06/30/2024 17:55)
results

(MsgRcvd 06/30/2024 18:09) Final

Test
CBC

Result

Flag

****(Reference)****

COMPLETE BLOOD COUNT

WBC	9.7		K/uL	(4.0 - 10.5)
RBC	4.43	L	K/uL	(4.50 - 5.70)
HEMOGLOBIN	14.2		g/dL	(13.5 - 17.2)
HEMATOCRIT	42.7		%	(35.0 - 65.0)
MCV	96		fL	(90 - 100)
MCH	32.1	H	pg	(27.0 - 31.2)
MCHC	33.3		g/dL	(32.0 - 35.4)
RDW	12.4		%	(12.0 - 14.8)
PLATELETS	162		K/uL	(142 - 424)
MPV	9.3		fL	(6.5 - 12.0)
%NEUT	77	H	%	(41 - 71)
%LYMPH	16	L	%	(22 - 44)
%MONO	5		%	(0 - 7)
%EOS	1		%	(0 - 4)
%BASO	1		%	(0 - 2)
#NEUT	7.4		K/uL	(1.8 - 7.8)
#LYMPH	1.6		K/uL	(1.0 - 4.8)
#MONO	0.4		K/uL	(0.0 - 0.8)
#EOS	0.1		K/uL	(0.0 - 0.4)
#BASO	0.1		K/uL	(0.0 - 0.2)
MANUAL DIFF	NOT INDICATED			
RBC MORPH	NOT INDICATED			

CMP: (COLL: 06/30/2024 17:55) (MsgRcvd 06/30/2024 18:23) Final results

Test	Result	Flag	**(Reference)**	
COMPREHENSIVE METABOLIC PANEL				
COMPREHENSIVE METABOLIC PANEL				
SODIUM	139.0		mEq/L	(137 - 145)
POTASSIUM	4.3		mEq/L	(3.4 - 5.0)
CHLORIDE	108	H	mEq/L	(98 - 107)
ECO2	25.0		mEq/L	(22.0 - 30.0)
ANION GAP	10	L	mEq/L	(12 - 18)
GLUCOSE	104		mg/dL	(75 - 106)
BUN	27	H	mg/dL	(9 - 20)
CREATININE	1.5		mg/dL	(0.8 - 1.5)
AGE	87		ys	
NON-AA GFR	47		mL/min	
AFR AMER GFR	57		mL/min	
CALCIUM	9.1		mg/dL	(8.4 - 10.2)
ALBUMIN	3.9		g/dL	(3.5 - 5.1)
TOTAL PROTEIN	7.4		g/dL	(6.3 - 8.2)
GLOBULIN	3.5		g/dL	(2.4 - 3.5)
A/G RATIO	1.1			(1.1 - 2.2)
SGOT/AST	42		U/L	(17 - 59)
SGPT/ALT	24		U/L	(21 - 72)
ALKALINE PHOS	93		U/L	(38 - 126)
TOTAL BILI	0.5		mg/dL	(0.2 - 1.3)

Lipase: (COLL: 06/30/2024 17:55) (MsgRcvd 06/30/2024 18:23) Final results

Test	Result	Flag	** (Reference) **
LIPASE	386	H	U/L (23 - 300)

Prothrombin Time (PT): (COLL: 06/30/2024 17:55) (MsgRcvd 06/30/2024 18:28) Final results

Test	Result	Flag	** (Reference) **
Protime	11.0		seconds(9.3 - 11.4)
INR	1.07		(0.90 - 1.11)

TIME W/ INR INTERPRETATION\par Therapeutic range for Coumadin and related oral anticoagulants.
 -International Normalized Ratio(INR): 2.0-3.0 for Venous Thrombosis, Pulmonary Embolus, Tissue heart valves, Acute MI, Atrial Fibrillation
 Valvular heart disease and recurrent Systemic Embolism.
 -International Normalized Ration(INR): 2.5-3.5 for Mechanical Prosthetic Valve.

APTT: (COLL: 06/30/2024 17:55) (MsgRcvd 06/30/2024 18:28) Final results

Test	Result	Flag	** (Reference) **
PTT	24.0	L	0 (25.0 - 34.6)

INTERPRETATION\par Critical results for patients not on therapy: >60 seconds
 Critical results for patients on therapy: >119 seconds
 Therapeutic range for patients on therapy: 41 - 54 seconds
 Coag studies from line draws may not be accurate due to Heparin and other interferences.

Lactic Acid: (COLL: 06/30/2024 17:55) (MsgRcvd 06/30/2024 18:23) Final results

Test	Result	Flag	** (Reference) **
LACTIC ACID	1.0		mmol/L (0.7 - 2.1)

CT Head wo IV Cont: (COLL: 06/30/2024 17:50) (MsgRcvd 06/30/2024 19:33) Final results

Exam
 CT HEAD W/O CONTRAST

Bear Valley Community Hospital
 Radiology Department
 Box 1649 - 41870 Garstin Drive
 Big Bear Lake, CA 92315
 909-878-8204 ? Fax: 909-878-8280

NAME: BEGLE, PETER
 DOB: 3/19/37

DATE OF EXAM: 6/30/24
 Patient No: 123464
 Extern Acct: E10279723
 Accsn: E500260920240630

Physician: HOYER, RILEY

EXAM: CT HEAD W/O CONTRAST

HISTORY: HEADACHE S/P TC

COMPARISON: CT HEAD W/O CONTRAST on 07/08/2022, CT HEAD W/O CONTRAST on 07/09/2021

TECHNIQUE: Axial images were obtained and reformatted in coronal and sagittal planes.

All CT scans at this medical facility are performed using dose modulation techniques as appropriate to a performed exam including the following: Automated exposure control was utilized; adjustment of the MA and/or KV according to patient size; and use of iterative reconstruction technique.

CT Dose: CTDI volume is 57.5 mGy. Dose-length product is 1039.3 mGy*cm

FINDINGS:

Supratentorial Region: No evidence for large acute territorial ischemia. No intracranial hemorrhage is noted.

Posterior Fossa: No acute abnormality.

Brainstem: Unremarkable.

Sellar/Suprasellar Region: Unremarkable.

Ventricles, Cisterns, Sulci: Age-appropriate.

Orbits: Unremarkable.

Paranasal Sinuses: Secretions noted in the right sphenoid sinus. A 1 cm mucous retention cyst is seen in the right maxillary sinus. Partial opacification of left posterior ethmoid air cells.

Mastoid Air Cells: Unremarkable.

Vasculature: Intracranial arterial calcified plaque formation noted.

Bones/Soft Tissues: No acute abnormality.

Other: None.

IMPRESSION:

No acute intracranial process.

Electronically signed by Sokhan Sanj, Maryam at 06/30/2024 07:31 PM PST

CT C-Spine wo IV Cont: (COLL: 06/30/2024 17:50) (MsgRcvd 06/30/2024 19:53) Final results

Exam
CT C-SPINE W/O CONTRAST

Bear Valley Community Hospital
Radiology Department
Box 1649 - 41870 Garstin Drive
Big Bear Lake, CA 92315
909-878-8204 ? Fax: 909-878-8280

NAME: BEGLE, PETER
DOB: 3/19/37

DATE OF EXAM: 6/30/24
Patient No: 123464
Extern Acct: E10279723

Physician: HOYER, RILEY

Accsn: E500261020240630

EXAM: CT C-SPINE W/O CONTRAST

INDICATION: PAIN

EXAM DATE: 06/30/2024 09:17 PM

COMPARISON: None

TECHNIQUE: Multiple axial CT images of the cervical spine were obtained using bone algorithm.

Axial and coronal reformatting was done. Bone and soft tissue windows were reviewed.

Radiation Dose Information:

CT Dose: CTDI volume is 9.8 mGy. Dose-length product is 228.8 mGy*cm

FINDINGS:

The cervical alignment is intact. No acute cervical spine fracture is identified. The vertebral body heights are intact. No suspicious osseous lesions are identified.

Moderate multilevel degenerative changes are identified.

There is no prevertebral soft tissue swelling. Multiple subcentimeter hypodensities in the bilateral thyroid lobes. These can be further evaluated with nonemergent ultrasound if clinically indicated. Moderate calcification at the bilateral carotid bulbs without any definitive evidence of stenosis. Partially visualized biapical lungs are clear.

IMPRESSION:

1. No evidence of acute cervical spine fracture or traumatic malalignment.
2. All CT scans at this medical facility are performed using dose modulation techniques as appropriate to a performed exam including the following: Automated exposure control was utilized; adjustment of the MA and/or KV according to patient size; and use of iterative reconstruction technique.

Electronically signed by Sadat, Sayed at 06/30/2024 07:51 PM PST

CT ABD/PEL w IV Cont: (COLL: 06/30/2024 17:50) (MsgRcvd 06/30/2024 20:15) Final results

Exam
CT ABD/PEL W/CONTRAST

Bear Valley Community Hospital
Radiology Department
Box 1649 - 41870 Garstin Drive
Big Bear Lake, CA 92315
909-878-8204 ? Fax: 909-878-8280

NAME: BEGLE, PETER
DOB: 3/19/37

DATE OF EXAM: 6/30/24
Patient No: 123464
Extern Acct: E10279723

Physician: HOYER, RILEY

Accsn: E500261120240630

Procedure: CT ABD/PEL W/CONTRAST 06/30/2024 09:37 PM

Indication: ABDOMINAL PAIN.

Comparison Study: Radiograph performed same day , pelvic CT scan dated 03/29/2024

Technique: Axial images were obtained and reformatted in coronal and sagittal planes.

All CT scans at this medical facility are performed using dose modulation techniques as appropriate to a performed exam including the following: Automated exposure control was utilized; adjustment of the MA and/or KV according to patient size; and use of iterative reconstruction technique.

CT Dose: CTDI volume is 4.7 mGy. Dose-length product is 224 mGy*cm

FINDINGS:

CT abdomen and pelvis:

Lung Bases: Unremarkable.

Hepatobiliary: The upper portion of the liver is out of field-of-view. Partially seen 1.9 cm cyst lobe, segment II. Few subcentimeter hypodense foci are too small to likely. Several large gallstones in gallbladder lumen without evidence of cholecystitis..

Spleen: Unremarkable.

Pancreas: Unremarkable.

Adrenal Glands: Unremarkable.

GU tract: The kidneys are normal in size bilaterally . Mild bilateral hydronephrosis. No hydroureter. No renal calculi are seen. A 2.4 cm exophytic cyst is seen arising from the lower pole of the left kidney. Mildly thickened trabecular bladder wall several small diverticula. Subcentimeter, 4 mm stone is seen in the left side of the bladder lumen.

GI tract: A small sliding hiatal hernia noted. No evidence of small bowel obstruction. Right hemicolectomy.

Lymphatics: No mesenteric, retroperitoneal or periportal lymphadenopathy.

Vasculature: Aorta is normal in caliber. Scattered calcified plaques are noted.

Pelvic Organs: Prostate is not enlarged . Large right hydrocele noted in the scrotal sac .

Bones/soft tissues: Acute , comminuted, nondisplaced fracture of the right iliac wing with a large hematoma in the right iliatus muscle noted. There is asymmetric enlargement of the right gluteal muscles reflecting strain without a discrete hematoma. Total right hip arthroplasty

without perihardware fracture. Heterotopic ossification on the lateral aspect of the right hip joint noted. The pectineal ring is intact. Suggestion of acute nondisplaced fracture of the right sacral ala. The sacroiliac joints are maintained.

Other: None.

CT lumbar spine:

Bones: Mild acute depression of the superior endplate of L5 with the proximally 20% loss of height without retropulsion. There is a subcentimeter avulsion fracture fragment at the anterior-superior corner of L5. No definite fracture of the posterior elements. Chronic degenerative grade 1 anterolisthesis of L4 on L5 and retrolisthesis of L5 on S1 without pars defects noted. Mild reduction of L5-S1 disc height. Discogenic endplate changes are seen at several levels. Evidence of central canal and neural foramina stenosis throughout the cervical spine due to degenerative disc disease.

Soft tissues: Paraspinal and prevertebral soft tissues are unremarkable.

IMPRESSION:

Acute comminuted nondisplaced right iliac wing fracture large right iliacus muscle intramuscular hematoma. Moderate right gluteal muscle strain without a discrete hematoma.

Findings suggestive of acute nondisplaced right sacral ala fracture.

Acute mild compression fracture of L5 with approximate 20% loss of height anteriorly without retropulsion.

Mild fullness of the bilateral renal pelvises that May represent hydronephrosis or parapelvic cysts.

Trabeculated bladder wall likely reflect longstanding outlet obstruction.

Cholelithiasis with no evidence for cholecystitis.

Right hemicolectomy.

More findings in the body of the report.

Electronically signed by Sokhan Sanj, Maryam at 06/30/2024 08:12 PM PST

Chest 1V Port: (COLL: 06/30/2024 17:50)
results

(MsgRcvd 06/30/2024 18:22) Final

Test
CHEST I v Port

Result

Flag

**** (Reference) ****

Bear Valley Community Hospital
Radiology Department
Box 1649 - 41870 Garstin Drive
Big Bear Lake, CA 92315
909-878-8204 ? Fax: 909-878-8280

NAME: BEGLE, PETER
DOB: 3/19/37

DATE OF EXAM: 6/30/24
Patient No: 123464
Extern Acct: E10279723
Accsn: E500261220240630

Physician: HOYER, RILEY

Procedure: CHEST 1v PORT 06/30/2024 07:54 PM

Indication: SHORTNESS OF BREATH.

Comparison: None

FINDINGS:

Lines and Tubes: None.

Cardiomediastinal: The heart is normal in size . Pulmonary vasculature is within normal limits .

Lungs: No focal pulmonary opacity is seen. The costophrenic angles are clear. No pneumothorax.

Bones/soft tissues: No acute abnormality is noted.

IMPRESSION:

1.

No acute cardiopulmonary disease. Hyperexpanded lungs.

Electronically signed by Sadat, Sayed at 06/30/2024 06:20 PM PST

Pelvis AP: (COLL: 06/30/2024 17:50)
results

(MsgRcvd 06/30/2024 18:29) Final

Test
PELVIS ROUTINE EXAM AP VIEW

Result

Flag

****(Reference)****

Bear Valley Community Hospital
Radiology Department
Box 1649 - 41870 Garstin Drive
Big Bear Lake, CA 92315
909-878-8204 ? Fax: 909-878-8280

NAME: BEGLE, PETER
DOB: 3/19/37

DATE OF EXAM: 6/30/24
Patient No: 123464
Extern Acct: E10279723
Accsn: E500261320240630

Physician: HOYER, RILEY

EXAM: PELVIS ROUTINE EXAM AP VIEW

CLINICAL INDICATION: PAIN

TECHNIQUE: 2 radiographic views of the pelvis were obtained.

Comparison: None

FINDINGS/IMPRESSION:

Status post total right hip arthroplasty. Intact orthopedic hardware without evidence of periprosthetic fractures or loosening. Mild-to-moderate degenerative changes of the left hip joint. Diffuse osteopenia. Surgical clips scattered over the right hemipelvis. Mild-to-moderate degenerative changes of the bilateral SI joints.

Electronically signed by Sadat, Sayed at 06/30/2024 06:26 PM PST

Clinical Report - Nurses

Bear Valley Community Health District

Emergency Department

41870 Garstin Drive, Big Bear Lake, CA 92315 909-866-6501

Patient: BEGLE, PETER

MRN: 123464 Acct#: 10279723

Sex: M DOB: 03/19/1937 Age: 87y

Arrival: 06/30/2024 17:36 Departure: Disposition: Transfer

Weight: 65 kg. Height/Length: 64 inches. BMI: 24.6

***This is a preliminary document and is subject to change**

TRIAGE

Arrived by EMS.

Triage time: 17:43 06/30/2024.

Chief Complaint: MOTOR VEHICLE COLLISION.

Location of injuries: right hip. --17:43 6/30/24 Barquin, Lisa, R.N.

No loss of consciousness. No headache, neck pain, back pain, numbness or weakness. --17:45 6/30/24 Barquin, Lisa, R.N.

Acuity: LEVEL 3. --17:46 6/30/24 Barquin, Lisa, R.N.

17:46 06/30/24. BP: 160/80. HR: 64. RR: 19. O2 saturation: 94%. Temp: 98.0 F. Pain level now 1/10. --17:47 6/30/24 Barquin, Lisa, R.N.

QUICK SEPSIS RELATED ORGAN FAILURE ASSESSMENT SCORE (qSOFA) SCORE: 0. No altered mentation (GCS less than 15), respiratory rate not greater than or equal to 22 breaths a minute or systolic blood pressure not less than or equal to 100 mmHg.

SEPSIS SCREEN: NEGATIVE. SIRS criteria negative. No possible sources of infection. --17:49 6/30/24 Barquin, Lisa, R.N.

Weight: 65 kg. Height/Length: 64 inches. BMI: 24.6. --17:48 6/30/24 Barquin, Lisa, R.N.

Medications

None. --17:44 6/30/24 Barquin, Lisa, R.N.

Allergies

NKDA. --17:44 6/30/24 Barquin, Lisa, R.N.

Patient: BEGLE, PETER

MRN: 123464

VisitID: 10279723

909-866-6501

87y, M

Clinical Report - Nurses

Bear Valley Community Health District

41870 Garstin Drive, Big Bear Lake, CA 92315

Registration Date/Time: 06/30/2024 17:36

History

PAST MEDICAL HX: No history of diabetes mellitus, hypertension, heart disease or lung disease.
SOCIAL HX: Never smoker. No alcohol use or drug use. He has not traveled outside the U.S.
Infectious disease exposure: No infectious disease exposure.

ABUSE ASSESSMENT: Abuse assessment. Abuse denied. No report of abuse. --17:45 6/30/24 Barquin, Lisa, R.N.

SELF HARM ASSESSMENT: Self harm assessment was performed. The patient answered "no" to the question(s) "Have you recently felt down, depressed, or hopeless?". --17:46 6/30/24 Barquin, Lisa, R.N.

NUTRITIONAL RISK ASSESSMENT: The nutritional risk assessment revealed no deficiencies.

FUNCTIONAL ASSESSMENT: Functional assessment: no impairments noted.

LEARNING NEEDS ASSESSMENT: The learning needs assessment revealed no barriers.

FALL RISK ASSESSMENT: Fall risk assessment completed. No risk factors identified.

SKIN INTEGRITY ASSESSMENT: Skin integrity risk assessment completed. No skin integrity risk identified. --17:46 6/30/24 Barquin, Lisa, R.N.

PHYSICAL ASSESSMENT

EXTREMITIES: Right hip: tenderness. --17:49 6/30/24 Barquin, Lisa, R.N.

GENERAL / NEURO / PSYCH: Alert. Oriented X 4. Appears in no acute distress. --17:49 6/30/24 Barquin, Lisa, R.N.

RESPIRATORY: Respirations not labored.

SKIN: Skin is warm and dry. --17:49 6/30/24 Barquin, Lisa, R.N.

NURSING PROGRESS NOTES

Call light placed in reach of patient. Side rails up x 2. Bed placed in lowest position. Brakes of bed on. --17:49 6/30/24 Barquin, Lisa, R.N.

17:46 06/30/2024 Site #1 started prior to arrival by EMS via IV in the left forearm with an 18g angiocath. --18:11 6/30/24 Zapotosky, Michael Zapotosky, R.N.

18:31 06/30/2024 Started 2 gm of Ancef (ceFAZolin Sodium) IVPB in bag #1 50 mL; at 50 mL/hr over 0.5 hour(s) via site #1. via dial-a-flow. Allergies verified and confirmed 5 rights. IV patency established. IV site checked: no pain, redness, or swelling. IV flushed thoroughly pre- and post-medication administration. Information reviewed with patient including reason for taking this medication, signs of allergic reaction and precautions. Verbalizes understanding. --18:36 6/30/24 Zapotosky, Michael Zapotosky, R.N.

18:37 06/30/2024 TDAP IM 0.5 mL given(Lot#: 333BM, expiration date: 8/17/2026, Manufacturer: GlaxoSmithKline). Given in the left deltoid. Allergies verified and confirmed 5 rights. Information reviewed with patient including reason for taking this medication, signs of allergic reaction and precautions. Verbalizes understanding. Vaccine information statement provided to the patient.

--18:37 6/30/24 Zapotosky, Michael Zapotosky, R.N.

Portable pelvis performed. Patient transported to CT by stretcher with tech and radiology tech.

--19:08 6/30/24 Hoogendyk, Kelly

Patient returned from CT by stretcher with tech. (Returned at 1930). --19:46 6/30/24 Hoogendyk, Kelly

Wound cleansed with sterile water. Wound irrigated with 150 mL using a 10 mL syringe; patient tolerated procedure well. --19:47 6/30/24 Hoogendyk, Kelly

19:49 06/30/2024 Ancef IVPB via IV site #1 Response: no adverse reaction. --19:49 6/30/24 Fogle, Joshua, R.N.

19:49 06/30/2024 TDAP IM Response: no adverse reaction. --19:49 6/30/24 Fogle, Joshua, R.N.

19:50 06/30/2024 Xylocaine Viscous * Topical 10 --19:51 6/30/24 Fogle, Joshua, R.N.

19:51 06/30/2024 LIDOCAINE 1% WITH EPINEPHRINE Injection Injectable 20 mL given. Allergies verified and confirmed 5 rights. Information reviewed with patient including reason for taking this medication, signs of allergic reaction and precautions. Verbalizes understanding. --19:51 6/30/24 Fogle, Joshua, R.N.

1/4" steri-strips applied to wound on right hand by tech. Dorsal right hand: applied clean dressing consisting of telfa pad. Secured with kerlix. --20:58 6/30/24 Hoogendyk, Kelly

21:21 06/30/2024 Gabapentin PO 300 mg given. Allergies verified and confirmed 5 rights. Information reviewed with patient including reason for taking this medication, signs of allergic reaction and precautions. Verbalizes understanding. --21:21 6/30/24 Fogle, Joshua, R.N.

This report is not final

Bear Valley Community Hospital
Radiology Department
Box 1649 - 41870 Garstin Drive
Big Bear Lake, CA 92315
909-878-8204 – Fax: 909-878-8280

NAME: BEGLE, PETER

DOB: 3/19/37

Physician: HOYER, RILEY

DATE OF EXAM: 6/30/24

Patient No: 123464

Extern Acct: E10279723

Accsn: E500261820240630

EXAM: FEMUR RT

CLINICAL INDICATION: PAIN

TECHNIQUE: 4 radiographic views of the right femur were obtained.

Comparison: None

FINDINGS/IMPRESSION:

There is no evidence of acute fracture or dislocation. Status post total right hip arthroplasty with intact appearing orthopedic hardware. Vascular calcifications.

The visualized joint space is well maintained.

Mild degenerative changes of the knee joint.

There is no radiopaque foreign body.

Electronically signed by Sadat, Sayed at 06/30/2024 07:20 PM PST

Bear Valley Community Hospital
Radiology Department
Box 1649 - 41870 Garstin Drive
Big Bear Lake, CA 92315
909-878-8204 – Fax: 909-878-8280

NAME: BEGLE, PETER

DOB: 3/19/37

Physician: HOYER, RILEY

DATE OF EXAM: 6/30/24

Patient No: 123464

Extern Acct: E10279723

Accsn: E500261720240630

EXAM: WRIST 3+VWS RT

CLINICAL INDICATION: PAIN

TECHNIQUE: 3 radiographic views of the right wrist were obtained.

Comparison: None

FINDINGS/IMPRESSION:

There is no evidence of acute fracture or dislocation.

The visualized joint space is well maintained.

The alignment is anatomical.

There is no radiopaque foreign body.

Electronically signed by Sadat, Sayed at 06/30/2024 07:19 PM PST

Bear Valley Community Hospital
Radiology Department
Box 1649 - 41870 Garstin Drive
Big Bear Lake, CA 92315
909-878-8204 – Fax: 909-878-8280

NAME: BEGLE, PETER

DOB: 3/19/37

Physician: HOYER, RILEY

DATE OF EXAM: 6/30/24

Patient No: 123464

Extern Acct: E10279723

Accsn: E500261520240630

EXAM: CT L-SPINE W/O CONTRAST

INDICATION: LOWER BACK PAIN- S/P TC

COMPARISON: None

TECHNIQUE: Multiple axial CT images of the lumbar spine were obtained using bone algorithm. Axial and coronal reformatting was done. Bone and soft tissue windows were reviewed.

Radiation Dose Information:

CT Dose: CTDI volume is 12.3 mGy. Dose-length product is 378.5 mGy*cm

FINDINGS:

Fracture of the superior endplate of L5 with minimal approximately 2 mm retropulsion. There is approximately 25% height loss . Partially visualized bilateral renal cysts. Grade 1 retrolisthesis of L5 in relation L4.

The disc spaces are relatively preserved. There is multilevel degenerative change of the spine, with disc space narrowing, subchondral sclerosis, and marginal osteophyte formation. Degenerative changes of the bilateral SI joints which include osteophytosis and osseous fusion.

IMPRESSION:

1. Acute wedge-shaped compression deformity involving the anterior vertebral body at L5 with approximately 25% height loss and minimal retropulsion.
2. Radiation optimization: All CT scans at this facility use at least one of these dose optimization techniques: automated exposure control mA and/or kV adjustment per patient size (includes targeted exams where dose is matched to clinical indication) or iterative reconstruction.

Electronically signed by Sadat, Sayed at 06/30/2024 08:18 PM PST

Bear Valley Community Hospital
Radiology Department
Box 1649 - 41870 Garstin Drive
Big Bear Lake, CA 92315
909-878-8204 – Fax: 909-878-8280

NAME: BEGLE, PETER

DOB: 3/19/37

Physician: HOYER, RILEY

DATE OF EXAM: 6/30/24

Patient No: 123464

Extern Acct: E10279723

Accsn: E500261620240630

EXAM: HAND 3+VWS RT

CLINICAL INDICATION: HANDS PAIN

TECHNIQUE: 3 radiographic views of the right hand were obtained.

Comparison: None

FINDINGS/IMPRESSION:

There is no evidence of acute fracture or dislocation.

Mild-to-moderate degenerative changes.

The alignment is anatomical.

There is no radiopaque foreign body.

Electronically signed by Sadat, Sayed at 06/30/2024 07:18 PM PST

Bear Valley Community Hospital
Radiology Department
Box 1649 - 41870 Garstin Drive
Big Bear Lake, CA 92315
909-878-8204 – Fax: 909-878-8280

NAME: BEGLE, PETER

DOB: 3/19/37

Physician: HOYER, RILEY

DATE OF EXAM: 6/30/24

Patient No: 123464

Extern Acct: E10279723

Accsn: E500261120240630

Procedure: CT ABD/PEL W/CONTRAST 06/30/2024 09:37 PM

Indication: ABDOMINAL PAIN.

Comparison Study: Radiograph performed same day , pelvic CT scan dated 03/29/2024

Technique: Axial images were obtained and reformatted in coronal and sagittal planes.

All CT scans at this medical facility are performed using dose modulation techniques as appropriate to a performed exam including the following: Automated exposure control was utilized; adjustment of the MA and/or KV according to patient size; and use of iterative reconstruction technique.

CT Dose: CTDI volume is 4.7 mGy. Dose-length product is 224 mGy*cm

FINDINGS:

CT abdomen and pelvis:

Lung Bases: Unremarkable.

Hepatobiliary: The upper portion of the liver is out of field-of-view. Partially seen 1.9 cm cyst lobe, segment II. Few subcentimeter hypodense foci are too small to likely. Several large gallstones in gallbladder lumen without evidence of cholecystitis..

Spleen: Unremarkable.

Pancreas: Unremarkable.

Adrenal Glands: Unremarkable.

GU tract: The kidneys are normal in size bilaterally . Mild bilateral hydronephrosis. No hydroureter. No renal calculi are seen. A 2.4 cm exophytic cyst is seen arising from the lower pole of the left kidney. Mildly

thickened trabecular bladder wall several small diverticula. Subcentimeter, 4 mm stone is seen in the left side of the bladder lumen.

GI tract: A small sliding hiatal hernia noted. No evidence of small bowel obstruction. Right hemicolectomy.

Lymphatics: No mesenteric, retroperitoneal or periportal lymphadenopathy.

Vasculature: Aorta is normal in caliber. Scattered calcified plaques are noted.

Pelvic Organs: Prostate is not enlarged . Large right hydrocele noted in the scrotal sac .

Bones/soft tissues: Acute , comminuted, nondisplaced fracture of the right iliac wing with a large hematoma in the right iliacus muscle noted. There is asymmetric enlargement of the right gluteal muscles reflecting strain without a discrete hematoma. Total right hip arthroplasty without perihardware fracture. Heterotopic ossification on the lateral aspect of the right hip joint noted. The pectineal ring is intact. Suggestion of acute nondisplaced fracture of the right sacral ala. The sacroiliac joints are maintained.

Other: None.

CT lumbar spine:

Bones: Mild acute depression of the superior endplate of L5 with the proximally 20% loss of height without retropulsion. There is a subcentimeter avulsion fracture fragment at the anterior-superior corner of L5.No definite fracture of the posterior elements. Chronic degenerative grade 1 anterolisthesis of L4 on L5 and retrolisthesis of L5 on S1 without pars defects noted. Mild reduction of L5-S1 disc height. Discogenic endplate changes are seen at several levels. Evidence of central canal and neural foramina stenosis throughout the cervical spine due to degenerative disc disease.

Soft tissues: Paraspinal and prevertebral soft tissues are unremarkable.

IMPRESSION:

Acute comminuted nondisplaced right iliac wing fracture large right iliacus muscle intramuscular hematoma. Moderate right gluteal muscle strain without a discrete hematoma.

Findings suggestive of acute nondisplaced right sacral ala fracture.

Acute mild compression fracture of L5 with approximate 20% loss of height anteriorly without retropulsion.

Mild fullness of the bilateral renal pelvises that May represent hydronephrosis or parapelvic cysts.

Trabeculated bladder wall likely reflect longstanding outlet obstruction.

Cholelithiasis with no evidence for cholecystitis.

Cholelithiasis with no evidence for cholecystitis.

Right hemicolectomy.

More findings in the body of the report.

Electronically signed by Sokhan Sanj, Maryam at 06/30/2024 08:12 PM PST

Bear Valley Community Hospital
Radiology Department
Box 1649 - 41870 Garstin Drive
Big Bear Lake, CA 92315
909-878-8204 – Fax: 909-878-8280

NAME: BEGLE, PETER

DOB: 3/19/37

Physician: HOYER, RILEY

DATE OF EXAM: 6/30/24

Patient No: 123464

Extern Acct: E10279723

Accsn: E500261020240630

EXAM: CT C-SPINE W/O CONTRAST

INDICATION: PAIN

EXAM DATE: 06/30/2024 09:17 PM

COMPARISON: None

TECHNIQUE: Multiple axial CT images of the cervical spine were obtained using bone algorithm. Axial and coronal reformatting was done. Bone and soft tissue windows were reviewed.

Radiation Dose Information:

CT Dose: CTDI volume is 9.8 mGy. Dose-length product is 228.8 mGy*cm

FINDINGS:

The cervical alignment is intact. No acute cervical spine fracture is identified. The vertebral body heights are intact. No suspicious osseous lesions are identified.

Moderate multilevel degenerative changes are identified.

There is no prevertebral soft tissue swelling. Multiple subcentimeter hypodensities in the bilateral thyroid lobes. These can be further evaluated with nonemergent ultrasound if clinically indicated. Moderate calcification at the bilateral carotid bulbs without any definitive evidence of stenosis. Partially visualized biapical lungs are clear.

IMPRESSION:

1. No evidence of acute cervical spine fracture or traumatic malalignment.
2. All CT scans at this medical facility are performed using dose modulation techniques as appropriate to a

performed exam including the following: Automated exposure control was utilized; adjustment of the MA and/or KV according to patient size; and use of iterative reconstruction technique.

Electronically signed by Sadat, Sayed at 06/30/2024 07:51 PM PST

Bear Valley Community Hospital
Radiology Department
Box 1649 - 41870 Garstin Drive
Big Bear Lake, CA 92315
909-878-8204 – Fax: 909-878-8280

NAME: BEGLE, PETER

DOB: 3/19/37

Physician: HOYER, RILEY

DATE OF EXAM: 6/30/24

Patient No: 123464

Extern Acct: E10279723

Accsn: E500261320240630

EXAM: PELVIS ROUTINE EXAM AP VIEW

CLINICAL INDICATION: PAIN

TECHNIQUE: 2 radiographic views of the pelvis were obtained.

Comparison: None

FINDINGS/IMPRESSION:

Status post total right hip arthroplasty. Intact orthopedic hardware without evidence of periprosthetic fractures or loosening. Mild-to-moderate degenerative changes of the left hip joint. Diffuse osteopenia. Surgical clips scattered over the right hemipelvis. Mild-to-moderate degenerative changes of the bilateral SI joints.

Electronically signed by Sadat, Sayed at 06/30/2024 06:26 PM PST

Bear Valley Community Hospital
Radiology Department
Box 1649 - 41870 Garstin Drive
Big Bear Lake, CA 92315
909-878-8204 – Fax: 909-878-8280

NAME: BEGLE, PETER

DOB: 3/19/37

Physician: HOYER, RILEY

DATE OF EXAM: 6/30/24

Patient No: 123464

Extern Acct: E10279723

Accsn: E500261220240630

Procedure: CHEST 1v PORT 06/30/2024 07:54 PM

Indication: SHORTNESS OF BREATH.

Comparison: None

FINDINGS:

Lines and Tubes: None.

Cardiomediastinal: The heart is normal in size . Pulmonary vasculature is within normal limits .

Lungs: No focal pulmonary opacity is seen. The costophrenic angles are clear. No pneumothorax.

Bones/soft tissues: No acute abnormality is noted.

IMPRESSION:

1.

No acute cardiopulmonary disease. Hyperexpanded lungs.

Electronically signed by Sadat, Sayed at 06/30/2024 06:20 PM PST

Bear Valley Community Hospital
Radiology Department
Box 1649 - 41870 Garstin Drive
Big Bear Lake, CA 92315
909-878-8204 – Fax: 909-878-8280

NAME: BEGLE, PETER

DOB: 3/19/37

Physician: HOYER, RILEY

DATE OF EXAM: 6/30/24

Patient No: 123464

Extern Acct: E10279723

Accsn: E500260920240630

EXAM: CT HEAD W/O CONTRAST

HISTORY: HEADACHE S/P TC

COMPARISON: CT HEAD W/O CONTRAST on 07/08/2022, CT HEAD W/O CONTRAST on 07/09/2021

TECHNIQUE: Axial images were obtained and reformatted in coronal and sagittal planes.

All CT scans at this medical facility are performed using dose modulation techniques as appropriate to a performed exam including the following: Automated exposure control was utilized; adjustment of the MA and/or KV according to patient size; and use of iterative reconstruction technique.

CT Dose: CTDI volume is 57.5 mGy. Dose-length product is 1039.3 mGy*cm

FINDINGS:

Supratentorial Region: No evidence for large acute territorial ischemia. No intracranial hemorrhage is noted.

Posterior Fossa: No acute abnormality.

Brainstem: Unremarkable.

Sellar/Suprasellar Region: Unremarkable.

Ventricles, Cisterns, Sulci: Age-appropriate.

Orbits: Unremarkable.

Paranasal Sinuses: Secretions noted in the right sphenoid sinus. A 1 cm mucous retention cyst is seen in the right maxillary sinus. Partial opacification of left posterior ethmoid air cells.

Mastoid Air Cells: Unremarkable.

Vasculature: Intracranial arterial calcified plaque formation noted.

Bones/Soft Tissues: No acute abnormality.

Other: None.

IMPRESSION:

No acute intracranial process.

Electronically signed by Sokhan Sanj, Maryam at 06/30/2024 07:31 PM PST

Bear Valley Community Hospital
Radiology Department
Box 1649 - 41870 Garstin Drive
Big Bear Lake, CA 92315
909-878-8204 – Fax: 909-878-8280

NAME: BEGLE, PETER

DOB: 3/19/37

Physician: HOYER, RILEY

DATE OF EXAM: 6/30/24

Patient No: 123464

Extern Acct: E10279723

Accsn: E500261920240630

EXAM: HIP UNILATERAL LIMITED RT

CLINICAL INDICATION: HIP PAIN

TECHNIQUE: 2 radiographic views of the right hip were obtained.

Comparison: HIP COMPLETE 2V RT on DOS: 9/14/23, HIP COMPLETE 2V RT on DOS: 2/28/24

FINDINGS/IMPRESSION:

There is no evidence of acute fracture or dislocation. Status post total right hip arthroplasty. Intact orthopedic hardware. Vascular calcifications.

The visualized joint space is well maintained.

The alignment is anatomical.

There is no radiopaque foreign body.

Electronically signed by Sadat, Sayed at 06/30/2024 07:21 PM PST

Bear Valley Community Hospital
Radiology Department
Box 1649 - 41870 Garstin Drive
Big Bear Lake, CA 92315
909-878-8204 – Fax: 909-878-8280

NAME: BEGLE, PETER

DOB: 3/19/37

Physician: HOYER, RILEY

DATE OF EXAM: 6/30/24

Patient No: 123464

Extern Acct: E10279723

Accsn: E500261920240630

EXAM: HIP UNILATERAL LIMITED RT

CLINICAL INDICATION: HIP PAIN

TECHNIQUE: 2 radiographic views of the right hip were obtained.

Comparison: HIP COMPLETE 2V RT on DOS: 9/14/23, HIP COMPLETE 2V RT on DOS: 2/28/24

FINDINGS/IMPRESSION:

There is no evidence of acute fracture or dislocation. Status post total right hip arthroplasty. Intact orthopedic hardware. Vascular calcifications.

The visualized joint space is well maintained.

The alignment is anatomical.

There is no radiopaque foreign body.

Electronically signed by Sadat, Sayed at 06/30/2024 07:21 PM PST